

From: AdventHealth Celebration Health

From: Christopher Everson To: Cigna Medicare (Healthspring) HHC & DME
 Phone: (407) 303-2200
 Fax to: (844) 215-4265 Attention: Main Fax
 Fax From:
 Return Fax:
 Comment: Please arrange HHC for RN and P.T. Thank you.

Regarding Patient: Mal,E
 SSN: XXX-XX-6286
 Member ID 1: 45000701
 Room: F354

The following documents are included in this fax:

Name	Pages
Home Health Orders - 12/19/2022 08:46:32 AM EST (-0500)	3
Home Health Packet - 12/19/2022	42

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**AdventHealth Celebration Health**

400 Celebration Place,
CELEBRATION, OSCEOLA, FL, 34747
Phone (407) 303-4000

Inbound Fax Cover Sheet**Patient Summary**

Patient:	Edna Maldonado	Episode:	5010088545281
MRN:	13740025	DOB:	08/12/1960

Document Abstract

Document Name: Home Health Orders
Prepared by: Katherine Shulterbrandt
Comments:

Document Contents

The attached document contains:

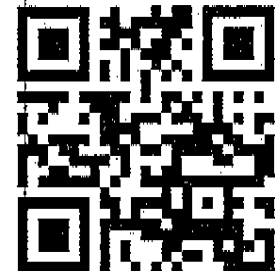
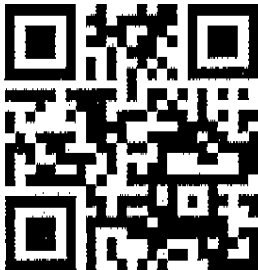
- ☒ Referral or Placement Information.
☐ Patient Record or View Only.

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- 1) Place this cover sheet on top of document set (ONE COVER SHEET REQUIRED PER PATIENT PACKET).
- 2) Fax complete package to (877) 659-7191.
- 3) PDF version of this document will be loaded into our secure Curaspan application after the fax transmission has been successfully completed. This upload and conversion process could take several minutes.



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Dr. Brad Homan
O: 321-939-0222 / F: 321-939-0225
 For any questions for Outpatient Discharges call Office
 Number listed Above.
 For questions for Inpatient Hospital Discharged
 Patients Call: Orthopedic Coordinator - Lisa Phillips
 407-303-4555

Home Health Nursing (Hip and Knee) 2X a week for two weeks and Physical Therapy (Knee 3X a week for two weeks, Hip safety check). Evaluate, follow up, and treat per procedure protocols below

1. Anticoagulation education, management, teach and train (per discharge medication reconciliation)
2. Incision care management, teach, and train per procedure protocol below
3. Patient to follow up with surgeon's office within 2-3 weeks (see discharge paperwork)

DME for home use: Front Wheeled Walker (FWW & 3:1 Commode (for knee/hip replacements)

Basic Patient Care:

Incision Care Encourage ankle pumps hourly Neurovascular Assessments Pain Management	Deep Breathing/Incentive Spirometer Bowel program Reinforce VTE prophylaxis
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Precautions/Restrictions:

<u>Total Hip Replacement (THA)</u> THA precautions No crossing legs and bending past 90 degrees	<u>Total Knee Replacement (TKA)</u> No squats or lunges Do not use CPM
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Incision Care/Dressings:

<u>Total Hip Replacement (THA)</u> • Notify surgeon if dressing becomes severely saturated or soiled • On POD 7, remove dressing, paint wound with betadine swab and apply new dressing. Do not remove staples, zipline. Second dressing will be removed at follow up appt. • While showering, cover the incision with a plastic wrap or trash bag. Remove plastic wrap/trash bag and wash previously covered area with CHG/Gentle soap and water and a clean wash cloth. • Bilateral below the knee TED hose to be worn 22 hours a day.	<u>Total Knee Replacement (TKA)</u> • Notify surgeon if dressing becomes severely saturated or soiled • On POD 7, remove dressing, paint wound with betadine swab and apply new dressing. Do not remove staples, zipline. Second dressing will be removed at follow up appt. • While showering, cover the incision with a plastic wrap or waterproof dressing. Remove plastic wrap/trash bag and wash previously covered area with CHG/Gentle soap and water and a clean wash cloth. • ACE wrap to remain on operated leg and below the knee TED hose to remain on unoperated leg for 22 hours a day.
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Physical Therapy: (Do Not use CPM)

<u>Total Hip Replacement (THA)</u> • Safety check • Continue exercises from hospital or PT established treatment plan. • Encourage ankle pumps • Ice gel packs to operative side after physical therapy and PRN	<u>Total Knee Replacement (TKA)</u> • FWB, FWW for ambulation, transition to cane per pt comfort level • Work to increase both active and passive ROM: Goal 90 degree flexion and 0 degree extension by 1 week p/o • Ice to operative side after physical therapy and PRN • Encourage ankle pumps
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Edna Maldonado
 Preferred name: Edna
 DOB: 8/12/1960 (62 yrs) Female
 DOS:
 MRN: 13740025 CSN: 5010088545281



Follow up Labs:

Follow up Appointments:

Procedure Left total knee Replacement

Date of Surgery 12/14/2022

Date of Discharge 12/19/2022

Signature: [Signature]

Date: 12/19/2022

Ordering and Following: Dr. Brad Homan # office: 321-939-0222 / fax: 321-939-0225 NPI# 1144254210

Edna Maldonado
Preferred name: Edna
DOB: 8/12/1960 (62 yrs) Female
DOS:
MRN: 13740025 CSN: 6010088545291



 Comments**Patient Demographics**

Name	Patient ID	SSN	Gender Identity	Birth Date
Maldonado, Edna	13740025	063-56-6286	Female	08/12/60 (62 yrs)
Address	Phone	Email		
14012 Colonial	407-406-2441	NATIVENYKR868@GMAIL.COM		
GRANDS BLVD	(H)			
APT 607	407-406-2441			
ORLANDO FL	(M)			
32837-4823				
Race	Occupation	Emp Status		
White	—	Unknown		
Reg Status	PCP			
Verified	Edgar Martin Ong, MD 407-304-1710			
Marital Status	Religion	Language		
Single	Unknown	English		
Emergency Contact 1	Emergency Contact 2		Emergency Contact 3	
Alejandro Savinon (Son)	Sorida Maldonado (Sister)		Eddie Rodriguez (Friend)	
407-923-2156 (M)	407-247-2850 (M)		347-528-6611 (M)	

Hospital Account

Name	Acct ID	Class	Status	Primary Coverage
Maldonado, Edna	1980000103799	Inpatient	Open	CIGNA MEDICARE - CIGNA MEDICARE HMO

Guarantor Account (for Hospital Account #1980000103799)

Name	Relation to Pt	Service Area	Active?	Acct Type
Maldonado, Edna	Self	AH	Yes	Personal/Family
Address	Phone		Gender	DOB
14012 Colonial GRANDS BLVD APT 607 ORLANDO, FL 32837-4823	407-406-2441(H)		Female	08/12/60
Emp Name	Emp Status	Occupation		
	Unknown			
Emp Address	Emp Phone			

Coverage Information (for Hospital Account #1980000103799)

F/O Payor/Plan	Subscriber DOB	Subscriber Sex	Precert #
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F/O Payor/Plan	Subscriber DOB	Subscriber Sex	Precert #
CIGNA MEDICARE/CIGNA MEDICARE HMO	08/12/60	F	
Subscriber		Relation to Pt	Subscriber #
Maldonado, Edna		Self	36147826
Grp #	Group Name		
H5410-024			
Address	Phone		
PO BOX 981706	800-230-6138		
EL PASO, TX 79998-1706			
Policy Number	Status	Effective Date	Benefits Phone
36147826	-		-
Subscriber Emp	Emp Phone	Emp Address	Auth/Cert
			NOT STARTED

COVID-19 Immunization Status

The follow-up could not be calculated because this patient does not have the relevant Health Maintenance topic. Refer to available guidelines.

Patient received PFIZER SARS-COV-2 VACCINATION on 9/1/2021.

Patient received PFIZER SARS-COV-2 VACCINATION on 8/11/2021.

History and Physical Notes

H&P by Tanveer Ahmed, MD at 12/14/2022 3:09 PM

Author: Tanveer Ahmed, MD	Service: Internal Medicine	Author Type: Physician
Filed: 12/14/2022 3:19 PM	Date of Service: 12/14/2022 3:09 PM	Status: Signed
Editor: Tanveer Ahmed, MD (Physician)		

HOSPITALIST HISTORY AND PHYSICAL

DATE OF ADMISSION: 12/14/2022

DATE OF SERVICE: 12/14/22

PCP:

Specialist: Dr homan Bred

CHIEF COMPLAINT:

Patient is admitted electively for left total knee arthroplasty failed outpatient medical treatment.

HISTORY OF PRESENT ILLNESS:

62-year-old female past medical history of degenerative joint disease status post back surgeries, history of osteoarthritis and failed outpatient medical treatment came to the direct admission from Dr.Homan office for left total knee arthroplasty underwent the procedure tolerated well. Patient was seen on to the floor alert awake complaining of headache secondary to her migraine flare-up, complaining of pain at the site of the surgery. Denies any chest pain shortness of breath palpitation. Denies any nausea vomiting denies any urinary symptoms of urgency frequency.

PAST MEDICAL HISTORY:

History of migraine headaches

Osteoarthritis of knees

Chronic back pain syndrome

Past Surgical History:

Procedure	Laterality	Date
• APPENDECTOMY		
• CHOLECYSTECTOMY		
• CT ANGIO CHEST W AND WO IV CONTRAST	N/A	04/28/2017
<i>CT ANGIO CHEST W AND WO IV CONTRAST</i>		
<i>4/28/2017 KSMERCONV</i>		
• CT ANGIO CHEST W AND WO IV CONTRAST	N/A	05/28/2022
<i>CT ANGIO CHEST W AND WO IV CONTRAST</i>		
<i>5/28/2022 KSMIPCONV</i>		
• CT ANGIO HEART CORONARY W IV CONTRAST	N/A	06/25/2017
<i>CT ANGIO HEART CORONARY W IV CONTRAST</i>		
<i>6/25/2017 AHCEL OP CONVERSION</i>		
• CT ANGIO HEART CORONARY W IV CONTRAST	N/A	02/19/2020
<i>CT ANGIO HEART CORONARY W IV CONTRAST</i>		
<i>2/19/2020 KSMIPCONV</i>		
• CT ANGIO HEART CORONARY W IV CONTRAST	N/A	10/29/2021
<i>CT ANGIO HEART CORONARY W IV CONTRAST</i>		
<i>10/29/2021 KSMOPCONV</i>		

Social History:

Tobacco: No history of smoking

Alcohol: No history of alcoholism

Drugs: No history of IV drug abuse

Family Medical History:

Father: Noncontributing

Mother: Non con to be

MEDICATION LIST:

No current facility-administered medications on file prior to encounter.

Current Outpatient Medications on File Prior to Encounter

Medication	Sig	Dispense	Refill
• ALPRAZolam (Xanax) 0.5 MG tablet	TAKE 1 TABLET BY MOUTH THREE TIMES A DAY FOR 3 DAYS AS NEEDED FOR ANXIETY		
• aspirin-acetaminophen-caffeine (Excedrin Migraine) 250-250-65 MG tablet	Take 1 tablet by mouth every 6 (six) hours if needed for headaches.		
• gabapentin (Neurontin) 800 MG tablet	TAKE 1 TABLET THREE TIMES DAILY MORNING NOON AND EVENING.		
• lactulose 20 gram/30 mL oral solution	Take 30 mL (20 g total) by mouth 1 (one) time each day if needed for constipation for up to 7 doses.	210 mL	0
• methocarbamol (Robaxin) 750 MG tablet	Take 750 mg by mouth in the morning and 750 mg at noon and 750 mg before bedtime.		
• traMADol (Ultram) 5 mg/mL oral suspension	100 mg, PO, BID, PRN as needed for pain, 0 Refill(s)		
• [EXPIRED] pantoprazole (Protonix) 40 MG EC tablet	Take 1 tablet (40 mg total) by mouth 1 (one) time each day. Do not crush, chew, or split.	30 tablet	0

ALLERGIES:

Meperidine hcl and Meperidine

REVIEW OF SYSTEMS:

10 Points review of system were normal except the positive finding as mentioned in history of

presenting illness.

PHYSICAL EXAMINATION:

Body mass index is 34.05 kg/m².

Physical Exam

Vitals and nursing note reviewed.

Constitutional:

Appearance: Normal appearance. He is normal weight.

HENT:

Head: Normocephalic and atraumatic.

Right Ear: External ear normal.

Left Ear: External ear normal.

Nose: Nose normal.

Mouth/Throat:

Mouth: Mucous membranes are moist.

Eyes:

Extraocular Movements: Extraocular movements intact.

Pupils: Pupils are equal, round, and reactive to light.

Cardiovascular:

Rate and Rhythm: Normal rate and regular rhythm.

Pulses: Normal pulses.

Heart sounds: Normal heart sounds. No murmur heard.

Pulmonary:

Effort: Pulmonary effort is normal.

Breath sounds: Normal breath sounds.

Abdominal:

General: Bowel sounds are normal.

Palpations: Abdomen is soft.

Musculoskeletal:

General: Normal range of motion.

Cervical back: Normal range of motion and neck supple.

Skin:

General: Skin is warm and dry.

Neurological:

General: No focal deficit present.

Mental Status: He is alert and oriented to person, place, and time.

Psychiatric:

Mood and Affect: Mood normal.

Behavior: Behavior normal.

LABS:

RADIOLOGY:

XR Knee 1-2 Views Left

Result Date: 12/14/2022

Well seated total knee arthroplasty. Created by: Jay R. Salmon, MD Signed by: Jay R.

Salmon, MD Signed on: 12/14/2022 11:37 EST Location: CHRR3

Assessment:

Status post left total knee arthroplasty postop day 0.

Osteoarthritis

Chronic back pain syndrome

Chronic migraine headaches

Obesity BMI 34.0

Plan:

PT evaluation and treatment

Diet as per surgery advise

Pain control per surgery recommendations

Resume home meds as appropriate

K and Mag protocol.

GI prophylaxis

DVT prophylaxis

acetaminophen, 1,000 mg, Oral, q6h

Or

acetaminophen, 650 mg, Oral, q6h

Or

acetaminophen, 650 mg, Rectal, q6h

acetaminophen, 650 mg, Oral, q6h

[START ON 12/15/2022] aspirin, 325 mg, Oral, Daily

ceFAZolin, 2 g, Intravenous, q8h

[START ON 12/15/2022] celecoxib, 200 mg, Oral, BID

gabapentin, 800 mg, Oral, TID

HYDROmorphine, , ,

methocarbamol, 500 mg, Oral, q8h

pantoprazole, 40 mg, Oral, Daily

promethazine, , ,

scopolamine, 1 patch, Transdermal, q24h

lactated Ringer's, 1,000 mL, Last Rate: 1,000 mL (12/14/22 1024)

lactated Ringer's, 125 mL/hr, Last Rate: 125 mL/hr (12/14/22 1223)

PRN medications: ALPRAZolam, diphenhydrAMINE, docusate sodium, HYDROcodone-acetaminophen, HYDROcodone-acetaminophen, ipratropium-albuterol, methocarbamol
FOLLOWED BY [START ON 12/16/2022] methocarbamol, ondansetron ODT **OR**
ondansetron **OR** ondansetron, oxyCODONE, senna-docusate, zolpidem

CODE STATUS: Full Code

I affirm that as of 12/14/2022 15:09, all documentation is accurate, including, if documented, Review of Systems, Physical Exam, Assessment, Plan, & any attestations, including anything

that was copied & pasted or pulled forward from templates or existing data.

Tanveer Ahmed, MD

Electronically signed by Tanveer Ahmed, MD at 12/14/2022 3:19 PM

H&P by Michael Earl Taylor, PA-C at 12/14/2022 6:53 AM

Author: Michael Earl Taylor, PA-C Service: Orthopedics Author Type: Physician Assistant
 Filed: 12/14/2022 6:55 AM Date of Service: 12/14/2022 6:53 AM Status: Signed

Editor: Michael Earl Taylor, PA-C (Physician Assistant) Cosigner: Bradley Michael Homan, DO at 12/14/2022 7:10 AM

History and Physical

Name: Edna Maldonado
DOB / AGE: 8/12/1960 / 62 y.o.
MRN: 13740025
CSN: 5010088545281
Date of Admission: 12/14/2022

Subjective

Chief Complaint
 Left knee pain

History of Present Illness

This is a 62 year old female who is following up for Chondromalacia. She was seen on August 11, 2022, at which time Order Plain X-ray/Interpretation was performed, Order MRI - Knee was performed, she was treated with Injection, Systemic and counseling knee djd was performed. We decided on the following plan on the right: MRI. We decided on the following plan on the left: MRI. Patient presents today for follow up of bilateral knee pain and MRI results. She states her pain continues and has been worsening. Patient is ambulating with cane assistance.

Past Medical History:

Diagnosis	Date
• Chronic kidney disease	
• Diverticulitis	
• Fibromyalgia	
• Neuropathy	
• Psoriasis	
• Rheumatoid arthritis (HCC)	

Past Surgical History:

Procedure	Laterality	Date
• APPENDECTOMY		
• CHOLECYSTECTOMY		

- CT ANGIO CHEST W N/A 04/28/2017
AND WO IV
CONTRAST
*CT ANGIO CHEST W AND WO IV CONTRAST
4/28/2017 KSMERCONV*
- CT ANGIO CHEST W N/A 05/28/2022
AND WO IV
CONTRAST
*CT ANGIO CHEST W AND WO IV CONTRAST
5/28/2022 KSMIPCONV*
- CT ANGIO HEART N/A 06/25/2017
CORONARY W IV
CONTRAST
*CT ANGIO HEART CORONARY W IV
CONTRAST 6/25/2017 AHCEL OP
CONVERSION*
- CT ANGIO HEART N/A 02/19/2020
CORONARY W IV
CONTRAST
*CT ANGIO HEART CORONARY W IV
CONTRAST 2/19/2020 KSMIPCONV*
- CT ANGIO HEART N/A 10/29/2021
CORONARY W IV
CONTRAST
*CT ANGIO HEART CORONARY W IV
CONTRAST 10/29/2021 KSMOPCONV*

Medications Prior to Admission

Medication	Sig	Dispense	Refill	Last Dose
• ALPRAZolam (Xanax) 0.5 MG tablet	TAKE 1 TABLET BY MOUTH THREE TIMES A DAY FOR 3 DAYS AS NEEDED FOR ANXIETY			Past Month
• aspirin-acetaminophen-caf feine (Excedrin Migraine) 250-250-65 MG tablet	Take 1 tablet by mouth every 6 (six) hours if needed for headaches.			Past Month
• gabapentin (Neurontin) 800 MG tablet	TAKE 1 TABLET THREE TIMES DAILY MORNING NOON AND EVENING.			12/13/2022
• lactulose 20 gram/30 mL oral solution	Take 30 mL (20 g total) by mouth 1 (one) time each day if needed for constipation for up to 7 doses.	210 mL	0	Past Week

• methocarbamol (Robaxin) 750 MG tablet	Take 750 mg by mouth in the morning and 750 mg at noon and 750 mg before bedtime.	12/13/2022
• traMADol (Ultram) 5 mg/mL oral suspension	100 mg, PO, BID, PRN as needed for pain, 0 Refill(s)	12/13/2022
• [EXPIRED] pantoprazole (Protonix) 40 MG EC tablet	Take 1 tablet (40 mg total) by mouth 1 (one) time each day. Do not crush, chew, or split.	30 tablet 0

Allergies

Allergen

- Meperidine Hcl
- Meperidine

DEMEROL

Reactions

Itching
Itching

No family history on file.

Social History

Socioeconomic History

- Marital status: Single

Tobacco Use

- Smoking status: Never
- Smokeless tobacco: Never

Vaping Use

- Vaping Use: Unknown

Substance and Sexual Activity

- Drug use: Never

Review of Systems

Per HPI

Objective

Vital Signs

12/14/22 0600

BP: 115/68

Pulse: 66

Resp: 18

Temp: 36.2 °C (97.1 °F)

SpO2: 96%

Weight: 98.6 kg (217 lb 6 oz)
Height: 1.702 m (5' 7")

Oxygen Therapy: None (Room air)

Body mass index is 34.05 kg/m².

Physical Exam

Skin: Left Knee: skin intact, no rashes or lesions.

No ligament instability Severe medial joint line tenderness and lateral joint line tenderness

Additional Notes: LEFT KNEE: No effusion Range of motion: 0-135° Moderate crepitation

Labs/Studies

No results found for this or any previous visit (from the past 24 hour(s)).

No imaging results in the past day.

Assessment

Active Problems:

There are no active Hospital Problems.

Left knee DJD

Plan

Left TKA

Electronically signed by Bradley Michael Homan, DO at 12/14/2022 7:10 AM

H&P by User Epic at 11/30/2022 4:38 PM

Author: User Epic	Service: —	Author Type: Physician
Filed: 11/30/2022 4:38 PM	Date of Service: 11/30/2022 4:38 PM	Status: Signed

Editor: Gretta Pena (Patient Care Tech)

History and Physical - Scan on 11/30/2022 4:38 PM: H&P - 11.15.22

Hospital Problems

Active Lines, Drains, and Airways

Peripheral intravenous line

Peripheral IV 12/17/22 Left;Posterior Forearm

Duration

2 days

Diet Orders

Adult diet Regular: General starting at 12/14 1152

Current Hospital Medications

acetaminophen (Tylenol) tablet 650 mg	650 mg, Oral, Every 6 hours, Max total dose of 4 grams per 24 hr.
ALPRAZolam (Xanax) tablet 0.5 mg	0.5 mg, Oral, Nightly PRN
aspirin tablet 325 mg	325 mg, Oral, Daily
benzonatate (Tessalon) capsule 100 mg	100 mg, Oral, 3 times daily PRN, Do not crush or chew.
budesonide-formoterol (Symbicort) 160-4.5 MCG/ACT inhaler 2 puff	2 puffs, Inhalation, 2 times daily RT, Rinse mouth with water after use to reduce aftertaste and incidence of candidiasis. Do not swallow.
butalbital-acetaminophen-caffeine 50-325-40 MG per tablet 1 tablet	1 tablet, Oral, Every 4 hours PRN
celecoxib (CeleBREX) capsule 200 mg	200 mg, Oral, 2 times daily
cetirizine (Zyrtec) tablet 10 mg	10 mg, Oral, Daily
diphenhydramine (Benadryl) capsule 50 mg	50 mg, Oral, Every 6 hours PRN
docusate sodium (Colace) capsule 100 mg	100 mg, Oral, 2 times daily PRN, Bowel Regimen - for prevention of constipation.
gabapentin (Neurontin) capsule 800 mg	800 mg, Oral, 3 times daily
guaifenesin-dextromethorphan (Robitussin DM) 100-10 MG/5ML syrup 5 mL	5 mL, Oral, Every 6 hours PRN
HYDROcodone-acetaminophen (Norco) 10-325 MG per tablet 1 tablet	1 tablet, Oral, Every 4 hours PRN, If inadequate response within 60 mins, contact provider if no further options ordered.
HYDROcodone-acetaminophen 5-325 MG tablet	1 tablet, Oral, Every 4 hours PRN, If inadequate response within 60 mins, contact provider if no further options ordered.
HYDROMorphone (Dilaudid) injection 0.5 mg	0.5 mg, Intravenous, Every 2 hour PRN
ipratropium-albuterol (Duo-Neb) 0.5-2.5 mg/3 mL nebulizer solution 3 mL	3 mL, Nebulization, Every 6 hours PRN
ipratropium-albuterol (Duo-Neb) 0.5-2.5 mg/3 mL nebulizer solution 3 mL	3 mL, Nebulization, Continuous, Every 20 mins, 3 dose/time, continuous treatment
lactated Ringer's infusion	125 mL/hr (125 mL/hr), Intravenous, Continuous @ 125 mL/hr
methocarbamol (Robaxin) tablet 500 mg	500 mg, Oral, Every 8 hours PRN, Start 8 hrs after last scheduled dose.
Linked Group 1: See attached printout for full Linked Orders Report.	
ondansetron (Zofran) injection 4 mg	4 mg, Intravenous, Every 8 hours PRN, (First Line Agent). Give IV if unable to take PO. If inadequate response within 60 minutes, proceed to second line agent or contact provider if no further options ordered.

Linked Group 2: See attached printout for full Linked Orders Report.

ondansetron (Zofran) injection 4 mg 4 mg, Intramuscular, Every 8 hours PRN, (First Line Agent). Give IM if unable to take PO and does not have IV access. If inadequate response within 60 minutes, proceed to second line agent or contact provider if no further options ordered.

Linked Group 2: See attached printout for full Linked Orders Report.

ondansetron ODT (Zofran-ODT) disintegrating tablet 8 mg 8 mg, Oral, Every 8 hours PRN, (First Line Agent). Patient should allow tablet to dissolve on tongue. Do not remove from blister pack until just before administering. If inadequate response within 60 minutes, proceed to second line agent or contact provider if no further options ordered.

Linked Group 2: See attached printout for full Linked Orders Report.

pantoprazole (Protonix) EC tablet 40 mg 40 mg, Oral, Daily, Do not crush, chew, or split.

senna-docusate (Peri-Colace) 8.6-50 MG per tablet 2 tablet 2 tablets, Oral, Daily PRN, (First Line Agent) for treatment of constipation - give scheduled if no bowel movement in past 24 hrs.

zolpidem (Ambien) tablet 2.5 mg 2.5 mg, Oral, Nightly PRN

➦ Height & Weight (last 3 days)

None

📋 Dietitian Nutrition Assessment

Flowsheet Row Most Recent Value

Nutrition Intake

Percent Meals Eaten (%) 100

Advance Care Planning Documents

Document Type	Status	Effective Date	Expiration Date	Received On	Description
Power of Attorney	Not Received				
Advanced Directive Patient Generated	Received				ANNUAL CONSENT

Physician Progress Notes (Notes for yesterday and today) Notes for yesterday and today

Progress Notes by Tanveer Ahmed, MD at 12/18/2022 4:14 PM

Author: Tanveer Ahmed, MD Service: Internal Medicine Author Type: Physician
 Filed: 12/18/2022 4:15 PM Date of Service: 12/18/2022 4:14 PM Status: Signed
 Editor: Tanveer Ahmed, MD (Physician)

PROGRESS NOTE

Name: Edna Maldonado
DOB / AGE: 8/12/1960 / 62 y.o.
MRN: 13740025

CSN: 5010088545281

Date of Admission: 12/14/2022

Date of service :12/18/2022

Subjective

Patient is seen and examined. Case discussed with patient and RN. Complaining of right-sided chest pain and cough. Assessment

Status post left total knee arthroplasty postop day 3

Osteoarthritis

Chronic back pain syndrome

Chronic migraine headaches

Obesity BMI 34.0

Chest pain and shortness of breath

Plan

DuoNeb nebulizer treatment Q 6 hourly p.r.n.

Pulmicort nebulizer treatment Q 12 hourly

Formoterol nebulizer treatment Q 12 hourly

Chest x-ray shows persistent elevation of hemidiaphragm right-side

Pulmonary consult for abnormal chest x-ray

CTA chest ruled out PE

Zyrtec 10 mg p.o. q.day

Incentive spirometry 10 times an hour

Fioricet 1-2 tablets p.o. Q 6 hourly p.r.n. for migraine headache

PT evaluation and treatment.

Current Facility-Administered Medications:

- acetaminophen (Tylenol) tablet 650 mg, 650 mg, Oral, q6h, Michael Earl Taylor, PA-C
- ALPRAZolam (Xanax) tablet 0.5 mg, 0.5 mg, Oral, Nightly PRN, Michael Earl Taylor, PA-C, 0.5 mg at 12/16/22 2350
- aspirin tablet 325 mg, 325 mg, Oral, Daily, Michael Earl Taylor, PA-C, 325 mg at 12/18/22 0901
- benzonatate (Tessalon) capsule 100 mg, 100 mg, Oral, TID PRN, Tanveer Ahmed, MD, 100 mg at 12/18/22 0906
- budesonide-formoterol (Symbicort) 160-4.5 MCG/ACT inhaler 2 puff, 2 puff, Inhalation, BID, Tanveer Ahmed, MD, 2 puff at 12/18/22 0902
- butalbital-acetaminophen-caffeine 50-325-40 MG per tablet 1 tablet, 1 tablet, Oral, q4h PRN, Tanveer Ahmed, MD, 1 tablet at 12/18/22 1050
- celecoxib (CeleBREX) capsule 200 mg, 200 mg, Oral, BID, Michael Earl Taylor, PA-C, 200 mg at 12/18/22 0901
- cetirizine (ZyrTEC) tablet 10 mg, 10 mg, Oral, Daily, Tanveer Ahmed, MD, 10 mg at 12/18/22 0902
- diphenhydrAMINE (Benadryl) capsule 50 mg, 50 mg, Oral, q6h PRN, Michael Earl Taylor, PA-C
- docusate sodium (Colace) capsule 100 mg, 100 mg, Oral, BID PRN, Michael Earl Taylor, PA-C

- gabapentin (Neurontin) capsule 800 mg, 800 mg, Oral, TID, Michael Earl Taylor, PA-C, 800 mg at 12/18/22 1308
- guaifenesin-dextromethorphan (Robitussin DM) 100-10 MG/5ML syrup 5 mL, 5 mL, Oral, q6h PRN, Tanveer Ahmed, MD, 5 mL at 12/16/22 0108
- HYDROcodone-acetaminophen (Norco) 10-325 MG per tablet 1 tablet, 1 tablet, Oral, q4h PRN, Michael Earl Taylor, PA-C, 1 tablet at 12/18/22 1047
- HYDROcodone-acetaminophen 5-325 MG tablet, 1 tablet, Oral, q4h PRN, Michael Earl Taylor, PA-C, 1 tablet at 12/18/22 0402
- HYDROMorphone (Dilaudid) injection 0.5 mg, 0.5 mg, Intravenous, q2h PRN, Michael Earl Taylor, PA-C, 0.5 mg at 12/18/22 0906
- ipratropium-albuterol (Duo-Neb) 0.5-2.5 mg/3 mL nebulizer solution 3 mL, 3 mL, Nebulization, q6h PRN, Tanveer Ahmed, MD, 3 mL at 12/15/22 1458
- ipratropium-albuterol (Duo-Neb) 0.5-2.5 mg/3 mL nebulizer solution 3 mL, 3 mL, Nebulization, Continuous, Tanveer Ahmed, MD, 3 mL at 12/16/22 1128
- lactated Ringer's infusion, 125 mL/hr, Intravenous, Continuous, Michael Earl Taylor, PA-C, Last Rate: 125 mL/hr at 12/14/22 1223, 125 mL/hr at 12/14/22 1223
- [COMPLETED] methocarbamol (Robaxin) tablet 500 mg, 500 mg, Oral, q8h, 500 mg at 12/16/22 0320 **FOLLOWED BY** methocarbamol (Robaxin) tablet 500 mg, 500 mg, Oral, q8h PRN, Michael Earl Taylor, PA-C, 500 mg at 12/17/22 1652
- ondansetron ODT (Zofran-ODT) disintegrating tablet 8 mg, 8 mg, Oral, q8h PRN **OR** ondansetron (Zofran) injection 4 mg, 4 mg, Intravenous, q8h PRN, 4 mg at 12/16/22 2052 **OR** ondansetron (Zofran) injection 4 mg, 4 mg, Intramuscular, q8h PRN, Michael Earl Taylor, PA-C
- pantoprazole (Protonix) EC tablet 40 mg, 40 mg, Oral, Daily, Michael Earl Taylor, PA-C, 40 mg at 12/18/22 0902
- senna-docusate (Peri-Colace) 8.6-50 MG per tablet 2 tablet, 2 tablet, Oral, Daily PRN, Michael Earl Taylor, PA-C
- zolpidem (Ambien) tablet 2.5 mg, 2.5 mg, Oral, Nightly PRN, Michael Earl Taylor, PA-C

Review of Systems

Fourteen point review systems reviewed and otherwise negative other than stated above.

Physical Exam

Vital Signs

	12/18/22 0512	12/18/22 0710	12/18/22 1047	12/18/22 1529
BP:		115/66		100/68
BP		Right arm		
Location:				
Pulse:		77		90
Resp:		16	16	
Temp:		36.6 °C (97.8 ° F)		36.8 °C (98.3 ° F)
SpO2:	92%	96%		(!) 88%

Body mass index is 34.05 kg/m².

General: No acute distress, alert, oriented.

Eye: Pupils are equal, round and reactive to light, Normal conjunctiva.

HENT: Normocephalic, Oral mucosa is moist.

Neck: Supple, Non-tender.

Respiratory: Lungs are clear to auscultation, Respirations are non-labored

Cardiovascular: Normal rate, Regular rhythm.

Gastrointestinal: Soft, Non-tender, Non-distended.

Musculoskeletal: Normal range of motion

Integumentary: Warm, Dry, Intact

Neurologic: Alert, Oriented, No focal deficits.

Psychiatric: Cooperative, Appropriate mood & affect.

Labs/Studies

Results from last 7 days

Lab	Units	12/17/22 1202	12/15/22 0540
SODIUM	mmol/L	138	142
POTASSIUM	mmol/L	3.9	4.2
CHLORIDE	mmol/L	101	106
CARBON DIOXIDE	mmol/L	30.0	30.0
BUN	mg/dL	18.0	17.0
CREATININE	mg/dL	0.57*	0.56*
CALCIUM	mg/dL	8.6	8.4*
PROTEIN TOTAL	g/dL	6.2*	--
BILIRUBIN TOTAL	mg/dL	0.20	--
ALKALINE PHOSPHATASE	U/L	77	--
ALT	U/L	28	--
AST	U/L	13	--
GLUCOSE	mg/dL	104*	114*

I affirm that as of 12/18/2022 16:14, all documentation is accurate, including, if documented, Review of Systems, Physical Exam, Assessment, Plan, & any attestations, including anything that was copied & pasted or pulled forward from templates or existing data.

Tanveer Ahmed, MD

Electronically signed by Tanveer Ahmed, MD on 12/18/2022 4:15 PM

Progress Notes by Fred Chukwuemeka Umeh, MD at 12/18/2022 11:27 AM

Author: Fred Chukwuemeka
Umeh, MD

Service: Pulmonology

Author Type: Physician

Filed: 12/18/2022 1:13 PM

Date of Service: 12/18/2022 11:27 AM
Status: Signed

Editor: Fred Chukwuemeka Umeh, MD (Physician)

**Florida Lung Asthma & Sleep Specialists
(407) 507-2615**

Name: Edna Maldonado

DOB / AGE: 8/12/1960 / 62 y.o.**MRN:** 13740025**CSN:** 5010088545281**Date of Admission:** 12/14/2022**Subjective**

This is a 62-year-old obese woman with fibromyalgia rheumatoid arthritis neuropathy and spinal stenosis. Patient underwent left knee arthroplasty on December 14, 2022. Patient was doing well postoperative but developed SOB and worsening surgical site pain. CXR was negative for acute findings. Pulmonary was consulted for evaluation and treatment of SOB. Currently on room air, saturation 92%. Complaints of right sided chest pain, which radiates to the back/shoulder. Has mild SOB.

interval history

12/18: CT angio of the chest without any pulmonary embolism elevation right hemidiaphragm with bibasilar atelectasis. Feeling slightly better

Objective**Vital Signs**

	12/18/22 0436	12/18/22 0512	12/18/22 0710	12/18/22 1047
BP:	(!) 101/59		115/66	
BP			Right arm	
Location:				
Pulse:	76		77	
Resp:			16	16
Temp:	36.7 °C (98.1 °		36.6 °C (97.8 °	
	F)		F)	
SpO2:	(!) 90%	92%	96%	

O2 Delivery Method: Nasal Cannula

O2 Flow Rate (L/min): 2 L/min

Body mass index is 34.05 kg/m².

Intake/Output Summary (Last 24 hours) at 12/18/2022 1127

Last data filed at 12/18/2022 0800

	Gross per 24 hour
Intake	1170 ml

Output —
Net 1170 ml

Physical Exam

General: No acute distress

HEENT: NC/AT, PERRL, mucus membranes moist

Neck: Supple, trachea midline

CVS: s1, s2 norma, RRR, Ø M/R/G

Lungs: equal expansion, Ø rales, Ø ronchi, Ø wheeze

Abdomen: Soft, nontender, no guarding

Ext: Ø clubbing, Ø cyanosis, Ø edema

Neuro: no focal motor or sensory deficits

Psych: appropriate mood and affect

Labs/Studies

Recent Results (from the past 24 hour(s))

CBC

Collection Time: 12/17/22 12:02 PM

Result	Value	Ref Range
WBC	11.00 (H)	4.40 - 10.50 10 ³ /uL
RBC	3.70 (L)	3.75 - 5.00 10 ⁶ /uL
Hemoglobin	10.6 (L)	11.4 - 14.7 g/dL
Hematocrit	33.1 (L)	34.3 - 45.5 %
MCV	89.5	80.5 - 99.8 fL
MCH	28.6	26.8 - 33.0 pg
MCHC	32.0	31.0 - 35.4 g/dL
RDW	14.6	11.7 - 14.7 %
Platelet Count	219	139 - 361 10 ³ /uL
MPV	10.8	9.7 - 12.5 fL

Comprehensive Metabolic Panel (CMP)

Collection Time: 12/17/22 12:02 PM

Result	Value	Ref Range
Sodium	138	135 - 145 mmol/L
Potassium	3.9	3.5 - 5.0 mmol/L
Chloride	101	98 - 110 mmol/L
Carbon Dioxide	30.0	24.0 - 32.0 mmol/L

Anion Gap	7	5 - 15 mmol/L
BUN	18.0	5.0 - 25.0 mg/dL
Creatinine	0.57 (L)	0.60 - 1.20 mg/dL
Glucose	104 (H)	70 - 100 mg/dL
Calcium	8.6	8.5 - 10.5 mg/dL
AST	13	5 - 46 U/L
ALT	28	4 - 51 U/L
Alkaline Phosphatase	77	35 - 104 U/L
Protein, Total	6.2 (L)	6.5 - 8.0 g/dL
Albumin	3.30	3.20 - 5.50 g/dL
Globulin	2.9	1.9 - 3.9 g/dL
A/G Ratio	1.1	1.1 - 2.2
Bilirubin, Total	0.20	0.10 - 1.50 mg/dL
eGFR	102.9	mL/min/ [1.73_m2]

Procalcitonin Test

Collection Time: 12/17/22 12:02 PM

Result	Value	Ref Range
Procalcitonin	0.11 (H)	<0.10 ng/mL

Extra Green Top

Collection Time: 12/17/22 12:02 PM

Result	Value	Ref Range
Extra Tube	Hold for add-ons.	

CT Angio Chest W IV Contrast

Result Date: 12/17/2022

1. Unremarkable CTA thoracic aorta without aneurysm or dissection. 2. Elevation of the right hemidiaphragm with bilateral basilar atelectasis. Created by: Francisco Contreras, MD Signed by: Francisco Contreras, MD Signed on: 12/17/2022 23:10 EST Location: OSRR37

Assessment

1. Postoperative pulmonary insufficiency
2. Postop atelectasis
3. Osteoarthritis- S/P left knee arthroplasty 12/14
4. Dyspnea r/o reactive airway
5. Chest pain
6. Elevated right hemidiaphragm r/o paralysis

Plan

CT angio of the chest without any evidence of PE, bibasilar atelectasis and elevated right hemidiaphragm.

Continue Symbicort b.i.d.

Encourage IS/ambulation

DVT prophylaxis with SCDs

will get sniff test to rule out diaphragm paralyses

Electronically signed by Fred Chukwuemeka Umeh, MD on 12/18/2022 1:13 PM

Wound Care (last 72 hours)

Wound/Ostomy

Row Name	12/19/22 0521	12/19/22 0020	12/18/22 2030	12/18/22 1700	12/18/22 1308
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Braden Scale

Sensory Perceptions	—	—	4	—	—
Moisture	—	—	4	—	—
Activity	—	—	3	—	—
Mobility	—	—	3	—	—
Nutrition	—	—	3	—	—
Friction and Shear	—	—	3	—	—
Braden Scale Score	—	—	20	—	—

Comfort and Environment Interventions

Row Name	12/19/22 0521	12/19/22 0020	12/18/22 2030	12/18/22 1700	12/18/22 1308
Additional Comfort/Environmental Interventions	—	—	Cold therapy	—	—
Cold Therapy	—	—	Applied (Comment) knee ice pack changed	—	—

Wound 12/14/22 Incision Knee Left;Anterior

Wound Properties	Date First Assessed: 12/14/22 Time First Assessed: 0819 Primary Wound Type: Incision Location: Knee Wound Location Orientation: Left;Anterior				
Site Assessment	Unable to assess	Unable to assess	Unable to assess	Unable to assess	Unable to assess dressing present
Peri-Wound Assessment	Unable to assess	Unable to assess	Unable to assess	Unable to assess	Unable to assess dressing present
Dressing	Other (Comment) aquacel AG and ACE wrap	—	Other (Comment) aquacel AG and ACE wrap	Other (Comment) acquacel ag and ace bandage	Other (Comment) acquacel ag and ace bandage
Dressing Changed	—	—	—	Changed	—
Dressing Status	Dry;Intact	Dry;Intact	Dry;Intact	Clean;Intact;Dry	Clean;Intact;Dry
Row Name	12/18/22 0741	12/18/22 0332	12/18/22 0020	12/17/22 2040	12/17/22 1600

Braden Scale

Sensory Perceptions	4	—	—	4	—
Moisture	4	—	—	4	—
Activity	3	—	—	3	—
Mobility	3	—	—	3	—
Nutrition	4	—	—	3	—
Friction and Shear	3	—	—	3	—
Braden Scale Score	21	—	—	20	—

Comfort and Environment Interventions

Comfort	—	—	—	Repositioned	—
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Wound 12/14/22 Incision Knee Left;Anterior

Wound Properties	Date First Assessed: 12/14/22 Time First Assessed: 0819 Primary Wound Type: Incision Location: Knee Wound Location Orientation: Left;Anterior				
Site Assessment	Unable to assess dressing present	Unable to assess dressing present	Unable to assess	Unable to assess dressing present	Other (Comment) dressing present - dry and intact

Row Name	12/18/22 0741	12/18/22 0332	12/18/22 0020	12/17/22 2040	12/17/22 1600
Peri-Wound Assessment	Unable to assess dressing present	Unable to assess dressing present	Unable to assess	Unable to assess dressing present	Clean;Intact
Dressing	Other (Comment) dressing acquacel ag and ace bandage	Other (Comment) dressing acquacel ag and ace bandage	—	Other (Comment) dressing acquacel ag and ace bandage	Other (Comment) dressing Aquacel AG / Ace wrap
Dressing Status	Clean;Intact;Dry	Clean;Intact;Dry	Clean;Intact;Dry	Clean;Intact;Dry	Clean;Dry;Intact
Row Name	12/17/22 0800	12/17/22 0411	12/17/22 0220	12/17/22 0020	12/16/22 2016

Braden Scale

Sensory Perceptions	3	—	—	—	4
Moisture	4	—	—	—	4
Activity	3	—	—	—	3
Mobility	3	—	—	—	3
Nutrition	3	—	—	—	3
Friction and Shear	3	—	—	—	2
Braden Scale Score	19	—	—	—	19

Comfort and Environment Interventions

Comfort	—	—	Gown changed	—	—
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Wound 12/14/22 Incision Knee Left;Anterior

Wound Properties	Date First Assessed: 12/14/22 Time First Assessed: 0819 Primary Wound Type: Incision Location: Knee Wound Location Orientation: Left;Anterior				
Site Assessment	Other (Comment) dressing present - dry and intact	Unable to assess dressing present	—	Unable to assess	Unable to assess dressing present
Peri-Wound Assessment	Clean;Intact	Unable to assess dressing present	—	Unable to assess	Unable to assess dressing present
Dressing	Other (Comment) Aquacel AG / Ace wrap	Other (Comment) dressing acquacel ag and ace bandage	—	—	Other (Comment) dressing acquacel ag and ace bandage
Dressing Status	Clean;Dry;Intact	Clean;Intact;Dry	—	Clean;Intact;Dry	Clean;Intact;Dry

Row Name	12/16/22 1345	12/16/22 0922
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Braden Scale

Sensory Perceptions	—	4
Moisture	—	4
Activity	—	3
Mobility	—	3

Row Name	12/16/22 1345	12/16/22 0922
Nutrition	—	3
Friction and Shear	—	3
Braden Scale Score	—	20

Wound 12/14/22 Incision Knee Left;Anterior

Wound Properties	Date First Assessed: 12/14/22 Time First Assessed: 0819 Primary Wound Type: Incision	
Location:	Knee Wound Location Orientation: Left;Anterior	
Site	Unable to assess	Unable to assess
Assessment	dressing present	dressing present
Peri-Wound Assessment	Unable to assess dressing present	Unable to assess dressing present
Dressing	Other (Comment) acquacel ag and ace bandage	Other (Comment) acquacel ag and ace bandage
Dressing Status	Clean;Intact;Dry	Clean;Intact;Dry

PT, OT & SLP Evaluation Notes

Evaluation by Maria E Gattorno, PT at 12/14/2022 4:00 PM

Version 1 of 1

Author: Maria E Gattorno, PT	Service: —	Author Type: Physical Therapist
Filed: 12/14/2022 7:17 PM	Date of Service: 12/14/2022 4:00 PM	Status: Signed
Editor: Maria E Gattorno, PT (Physical Therapist)		

Inpatient Physical Therapy Evaluation

Name: Edna Maldonado
DOB / AGE: 8/12/1960 / 62 y.o.
MRN: 13740025
CSN: 5010088545281

Documentation Type: Initial Evaluation

Subjective

Patient Complaints: Numbness/tingling, Dizziness, Pain
 Patient Subjective Comments: "I cannt walk today, no way"/agreeable to PT
 Patient Stated Goals: "I want to go home "
 Pain Adequately Managed for Therapy Participation: Pain management not adequate to complete therapy session
 Active Problems:
 There are no active Hospital Problems.

Past Medical History:

Diagnosis Date

- Chronic kidney disease
- Diverticulitis
- Fibromyalgia
- Neuropathy
- Psoriasis
- Rheumatoid arthritis (HCC)

Past Surgical History:

Procedure Laterality Date

- APPENDECTOMY
- CHOLECYSTECTOMY
- CT ANGIO CHEST W AND WO IV CONTRAST
CT ANGIO CHEST W AND WO IV CONTRAST
4/28/2017 KSMERCONV
- CT ANGIO CHEST W AND WO IV CONTRAST
CT ANGIO CHEST W AND WO IV CONTRAST
5/28/2022 KSMIPCONV
- CT ANGIO HEART CORONARY W IV CONTRAST
CT ANGIO HEART CORONARY W IV CONTRAST
6/25/2017 AHCEL OP CONVERSION
- CT ANGIO HEART CORONARY W IV CONTRAST
CT ANGIO HEART CORONARY W IV CONTRAST
2/19/2020 KSMIPCONV
- CT ANGIO HEART CORONARY W IV CONTRAST
CT ANGIO HEART CORONARY W IV CONTRAST
10/29/2021 KSMOPCONV

History / Personal Factors

Significant Hospital Course Details: 62 y/o female pt admitted to AHC 12/14/22 for elective L TKA by Dr.Homan.(PT 12/14/22)

of Comorbidities/Personal Factors Impacting Plan of Care: 1-2 personal factors and/or

comorbidities

Precautions

Rehab Precautions/Restrictions: Yes

Medical Precautions: Fall Risk

Home Living

Type of Residence: Private residence

Type of Home: Apartment

Lives with: Adult children

Home Layout: One level

Home Access: Level entry

Bathroom Shower/Tub: Walk-in shower

Bathroom Toilet: Standard

Home Living Comments: pt indicates she lives on first floor apt

Prior Function

Level of Independence: Independent functional transfers, Independent with household ambulation

Premorbid Modified Rankin Score: Slight disability. Able to look after own affairs without assistance, but unable to carry out all previous activities.

Receives Help From: Family

Functional Device(s) Used Prior: Cane

Objective

Family/Caregiver Present: No

Nursing Pre-Treatment Communication: RN Wanda clear for PT IE

Post Treatment Communication: handsoff communication completed

Activity Tolerance

Endurance: Tolerates 10 - 20 min exercise with multiple rests

Early Mobility/Exercise Safety Screen: Proceed with mobilization - No exclusion criteria met

Cognition

Overall Cognitive Status: Within Functional Limits

Arousal / Alertness: Alert, Awake

Orientation Level: Oriented X4

Following Commands: Follows all commands and directions without difficulty

Safety Judgment: Decreased awareness of need for assistance, Decreased awareness of need for safety

Awareness of Errors: Assistance required to correct errors made, Assistance required to identify errors made

Perception

Perception: WFL

Sensation

Light Touch: Partial deficits in the LLE

Sensation Comments: pt reports decreased sensation L LE

Static Sitting Balance

Static Sitting Balance Grade: Good: Able to maintain balance against moderate resistance

Dynamic Sitting Balance

Dynamic Sitting Balance Grade: Good: Able to sit unsupported & weight shift across midline moderately

Static Standing Balance

Static Standing Balance Grade: Fair: Able to stand w/o UE support & w/o loss of balance for 1-2 minutes

Static Standing-Comment/Number of Minutes: w/FWW

Dynamic Standing Balance

Dynamic Standing Balance Grade: Fair: Able to stand unsupported, weight shift & reach to same side, difficulty crossing midline

Dynamic Standing-Comments: w/FWW

Postural Control

Postural Control: Within Functional Limits

Coordination

Movements are Fluid and Coordinated: Yes

Functional Mobility:

Modified Rankin Score: Moderately severe disability. Unable to attend to own bodily needs without assistance or unable to walk unassisted.

Bed Mobility

Bed Mobility From: Supine

Bed Mobility Type: To

Bed Mobility To: Side lying-right, Side lying-left

Level of Assistance: Minimal assist

Bed Mobility Comments: vc's to assist with bedrails

Transfers

Sit to Stand Assist: Minimal assist

Sit to Stand Transfer Device: Walker - front wheel

Bed/Chair/Wheelchair Transfer Assist: Minimal assist

Bed/Chair/WC Transfer Device: Walker - two wheel

Technique: Stand pivot

Comments: stand pivot to BSC and BTB/ pt doing NWB L LE despite education

Ambulation

Gait Comments: no functional gait tolerated today

Extremity Assessments:

RUE Assessment: Within Functional Limits

LUE Assessment: Within Functional Limits

RLE Assessment: Within Functional Limits

LLE Assessment: WFL except

AROM LLE (degrees)

L Knee Flexion 0-140: 75

L Knee Extension (0): -10

IP PT Additional Assessments:

PT Functional Outcomes

No documentation.

and

Treatment Activities

PT Exercises

Exercise Activity 1: antiembolics

Activity Performance 1: 10

Exercise Activity 2: TKA medbridge

Activity Performance 2: 10

Therapeutic Activity

Therapeutic Activity 1: bed mobility

Therapeutic Activity 2: EOB sitting balance/tolerance

Therapeutic Activity 3: transfer OOB to BSC and BTB

Therapeutic Activity 4: fall prevention education

Therapeutic Activity 5: HEP education

IP PT Evaluation Minutes

PT Evaluation (Moderate Complexity) Minutes: 1515

IP PT Therapeutic Interventions Minutes

Therapeutic Activity Minutes: 15

Therapeutic Exercise Minutes: 15

Assessment

Pt s/p L TKA today, she presents with overall decreased functional mobility and safety, related to acuity of procedure, able to participate on bed mobility, EOB sitting balance/tolerance and standing pivot transfer to BSC and BTB, pt doing NWB L LE despite education, safety

education and exercise program completed as well, medbridge handout provided, she will be f/u 14x week to progress with goals as tolerated, SNF for short term rehab recommended upon DC since no functional gait tolerated today, pending progress.

Impairments Noted (PT): Balance, Endurance, Judgement, Mobility, Pain, Range of motion, Safety awareness, Sensation

Evaluation/Treatment Tolerance: Treatment limited due to medical status

Motivation/Participation Level: Decreased motivation, Effort level inconsistent

Prognosis: Fair

Patient's Support Structure: Fair

Barriers to Discharge: needs to be Mod Ind with AD for transfers and household distances

PT Goals:

Therapy Goals Set With: Patient

PT Target Goal Date: 12/21/22

Bed Mobility Goal 1 (PT)

Activity/Assistive Device (Bed Mobility Goal 1, PT): bed mobility activities, all

Independence Level/Cues Needed (Bed Mobility Goal 1, PT): independent

Transfer Goal 1 (PT)

Activity/Assistive Device (Transfer Goal 1, PT): transfers, all

Independence Level/Cues Needed (Transfer Goal 1, PT): modified independence

Gait Training Goal 1 (PT)

Activity/Assistive Device (Gait Training Goal 1, PT): gait (walking locomotion), assistive device use

Independence Level (Gait Training Goal 1, PT): modified independence

Distance (Gait Training Goal 1, PT): 300

Strength Goal 1 (PT)

Strength Goal 1 (PT): pt to complete TKA phase I medbridge Ind

Safety Awareness Goal 1 (PT)

Activity (Safety Awareness Goal 1, PT): awareness of need for assistance, insight into deficits/self-awareness, safe use of assistive device/equipment, follow through of safety precautions

Independence/Cues/Accuracy (Safety Awareness Goal 1, PT): independent

PT Evaluation Complexity

of Comorbidities/Personal Factors Impacting Plan of Care: 1-2 personal factors and/or comorbidities

Examination of Body Systems: 3 or more elements

Clinical Presentation: Evolving with changing characteristics

Clinical Decision Making: Moderate complexity using standard pt assessment instrument &/or measurable assessment of functional outcome

Calculated Eval Type: 2

Charting Based Suggested Eval Complexity: Moderate Complexity

Plan

PT Plan: Continue skilled PT

PT Treatment Activities/Interventions: Gait, Therapeutic activities, Therapeutic exercises, Transfers

PT Treatment Focus: Education-patient, Energy conservation, Home exercise program, Home management

PT Frequency: 14x/wk

Discharge Dispo/Follow-up Recommendations: Skilled nursing facility

Post D/C Follow-up Therapy Service Recommendations: Physical Therapy

D/C Home Planning: pt needs to be Mod Ind with AD for transfers and household distances

Functional Mobility Equipment Recommended for D/C: Walker - front wheel

Adaptive Equipment Recommended for D/C: None/has all needed equipment

Physical Therapy Evaluation

Electronically signed by Maria E Gattorno, PT on 12/14/2022 7:17 PM

PT, OT & SLP Progress Notes (Notes from 12/17/22 through 12/19/22)

Notes from 12/17/22 through 12/19/22

Progress Notes by Steven J Kerbs, PT at 12/18/2022 2:05 PM

Author: Steven J Kerbs, PT

Service: —

Author Type: Physical Therapist

Filed: 12/18/2022 3:20 PM

Date of Service: 12/18/2022 2:05 PM

Status: Signed

PM

Editor: Steven J Kerbs, PT (Physical Therapist)

Inpatient Physical Therapy Treatment Note

Name: Edna Maldonado

MRN/CSN: 13740025 / 5010088545281

DOB / AGE: 8/12/1960 / 62 y.o.

Documentation Type: Treatment

Subjective

Patient Complaints: Pain, Weakness

Patient Subjective Comments: no other issues

Pain Adequately Managed for Therapy Participation: Pain management adequate throughout therapy session

Active Problems:

There are no active Hospital Problems.

Significant Hospital Course Details: 62 y/o female pt admitted to AHC 12/14/22 for elective L TKA by Dr.Homan.(PT 12/14/22)

Precautions

Rehab Precautions/Restrictions: Yes

LE Weight Bearing Status: WBAT

Medical Precautions: Fall Risk

Objective

Family/Caregiver Present: No

Nursing Pre-Treatment Communication: RN ok to see pt

Post Treatment Communication: RN handoff completed

PT Exercises

Exercise Activity 1: Review of HEP (ankle pumps, heel slides, SAQ, and QS to be performed 10x B LE at least once every waking hour.)

Therapeutic Activity

Therapeutic Activity 1: HEP education

Therapeutic Activity 2: bed mobility

Therapeutic Activity 3: transfers

Therapeutic Activity 4: fall prevention/home safety

Postural Control

Postural Control: Within Functional Limits

Functional Mobility:

Modified Rankin Score: Moderately severe disability. Unable to attend to own bodily needs without assistance or unable to walk unassisted.

Bed Mobility

Level of Assistance: Contact guard

Transfers

Sit to Stand Assist: Contact guard

Sit to Stand Transfer Device: Walker - front wheel

Bed/Chair/Wheelchair Transfer Assist: Contact guard

Bed/Chair/WC Transfer Device: Walker - two wheel

Technique: Stand to sit, Sit to stand, Walking

Ambulation

Distance (ft): 25

Assistance: Contact guard
Device: Walker - front wheel

IP PT Additional Assessments:

PT Functional Outcomes

Row Name 12/18/22 1405

PT Outcome/Test Tool Selection

Balance / Gait / Mobility 6 Clicks - 5 Items
(A-L)

AM-PAC 6-Clicks Basic Mobility (5 ITEMS) IP Short Form v2

Moving from lying on back to sitting on side of the bed?	3
Turning over in bed (including adjusting bedclothes, sheets & blankets)?	3
Sitting down on and standing up from a chair with arms (eg. wheelchair, bedside, commode)?	3
Moving to and from a bed to a chair (including a wheelchair)?	3
To walk in hospital room?	3
AM-PAC Raw Score (5-Items)	15
AM-PAC Standardized T Score (5-Items)	38.88

IP PT Therapeutic Interventions Minutes
Therapeutic Activity Minutes: 25

Assessment

Pt w/ attempted x2 w/ multiple excuses for not wanting to mobilize, educated, went to bathroom and back to bed. Pt is not candidate for skilled SNF. Has son at home to assist, DC to home w/ HH PT/OT prn. Pt would benefit from continued skilled services d/t aforementioned deficits

Impairments Noted (PT): Balance, Endurance, Mobility, Range of motion, Safety awareness, Pain, Strength

Evaluation/Treatment Tolerance: Limited by motivation

Motivation/Participation Level: Effort level decreased

Barriers to Discharge: needs to be Mod Ind with AD for transfers and household distances

PT Goals:

PT Target Goal Date: 12/21/22

Overall Progress Toward PT Goals: Goals reviewed-remain appropriate

Bed Mobility Goal 1 (PT)

Activity/Assistive Device (Bed Mobility Goal 1, PT): bed mobility activities, all

Independence Level/Cues Needed (Bed Mobility Goal 1, PT): independent

Transfer Goal 1 (PT)

Activity/Assistive Device (Transfer Goal 1, PT): transfers, all

Independence Level/Cues Needed (Transfer Goal 1, PT): modified independence

Gait Training Goal 1 (PT)

Activity/Assistive Device (Gait Training Goal 1, PT): gait (walking locomotion), assistive device use

Independence Level (Gait Training Goal 1, PT): modified independence

Distance (Gait Training Goal 1, PT): 300

ROM Goal 1 (PT)

ROM Goal 1 (PT): L knee 0-90 degrees

Strength Goal 1 (PT)

Strength Goal 1 (PT): pt to complete TKA phase I medbridge Ind

Safety Awareness Goal 1 (PT)

Activity (Safety Awareness Goal 1, PT): awareness of need for assistance, insight into deficits/self-awareness, safe use of assistive device/equipment, follow through of safety precautions

Independence/Cues/Accuracy (Safety Awareness Goal 1, PT): independent

Problem Specific Goal 1 (PT)

Problem Specific Goal 1 (PT): TKA HEP

Strategies/Barriers (Problem Specific Goal 1, PT): indep via medbridge

Plan

PT Plan: Continue skilled PT

PT Treatment Activities/Interventions: Adaptive/assistive equipment, Bed mobility, Gait, Range of motion, Therapeutic activities, Therapeutic exercises, Transfers

PT Treatment Focus: Body mechanics, Education-patient, Energy conservation, Home exercise program, Posture, Safety

PT Frequency: 7x/wk

Discharge Dispo/Follow-up Recommendations: Home with home care services

Post D/C Follow-up Therapy Service Recommendations: Physical Therapy

D/C Home Planning: pt needs to be Mod Ind with AD for transfers and household distances

Functional Mobility Equipment Recommended for D/C: None/has all needed equipment

Adaptive Equipment Recommended for D/C: None/has all needed equipment

Physical Therapy Visit

Electronically signed by Steven J Kerbs, PT on 12/18/2022 3:20 PM

Progress Notes by Adelaida I Ortiz, PT at 12/17/2022 3:44 PM

Author: Adelaida I Ortiz, PT

Service: —

Author Type: Physical Therapist

Filed: 12/17/2022 5:44 PM

Date of Service: 12/17/2022 3:44 PM
Status: Signed

Editor: Adelaida I Ortiz, PT (Physical Therapist)

Inpatient Physical Therapy Treatment Note

Name: Edna Maldonado

MRN/CSN: 13740025 / 5010088545281

DOB / AGE: 8/12/1960 / 62 y.o.

Documentation Type: Treatment

Subjective

Patient Complaints: Dizziness

Patient Subjective Comments: "I feel weird", pt consented to PT

Pain Adequately Managed for Therapy Participation: Pain management adequate throughout therapy session

Active Problems:

There are no active Hospital Problems.

Significant Hospital Course Details: 62 y/o female pt admitted to AHC 12/14/22 for elective L TKA by Dr.Homan.(PT 12/14/22)

Precautions

Rehab Precautions/Restrictions: Yes

LE Weight Bearing Status: WBAT

Medical Precautions: Fall Risk

Objective

Family/Caregiver Present: No

Nursing Pre-Treatment Communication: RN Kerry cleared for treatment

Post Treatment Communication: RN Kerry notified pt back in bed with all needs within reach

Activity Tolerance

Endurance: Tolerates 10 - 20 min exercise with multiple rests

Early Mobility/Exercise Safety Screen: Proceed with mobilization - No exclusion criteria met

Cognition

Overall Cognitive Status: Within Functional Limits

Arousal / Alertness: Drowsy, Awake

Orientation Level: Oriented X4

Following Commands: Follows all commands and directions without difficulty

Safety Judgment: Decreased awareness of need for assistance, Decreased awareness of need for safety

Awareness of Errors: Assistance required to identify errors made

Deficit Awareness: Fully aware of deficits

Attention Span: Appears intact

PT Exercises

Exercise Activity 1: Review of HEP (ankle pumps, heel slides, SAQ, and QS to be performed 10x B LE at least once every waking hour.)

Therapeutic Activity

Therapeutic Activity 1: HEP education

Therapeutic Activity 2: bed mobility

Therapeutic Activity 3: transfers

Therapeutic Activity 4: fall prevention/home safety

Static Sitting Balance

Static Sitting-Balance Support: No upper extremity supported

Static Sitting Balance Grade: Good: Able to maintain balance against moderate resistance

Dynamic Sitting Balance

Dynamic Sitting-Balance Support: No upper extremity supported

Dynamic Sitting Balance Grade: Good: Able to sit unsupported & weight shift across midline moderately

Static Standing Balance

Static Standing-Balance Support: Bilateral upper extremity supported

Static Standing Balance Grade: Fair: Able to stand w/o UE support & w/o loss of balance for 1-2 minutes

Static Standing-Comment/Number of Minutes: FWW

Dynamic Standing Balance

Dynamic Standing-Balance Support: Bilateral upper extremity supported

Dynamic Standing Balance Grade: Fair: Able to stand unsupported, weight shift & reach to same side, difficulty crossing midline

Dynamic Standing-Comments: FWW

Postural Control

Postural Control: Within Functional Limits

Functional Mobility:

Modified Rankin Score: Moderately severe disability. Unable to attend to own bodily needs without assistance or unable to walk unassisted.

Bed Mobility

Bed Mobility From: Supine

Bed Mobility Type: To and from

Bed Mobility To: Sit edge of bed

Level of Assistance: Contact guard

Bed Mobility Comments: Min A with L LE

Transfers

Sit to Stand Assist: Contact guard

Sit to Stand Transfer Device: Walker - front wheel

Bed/Chair/Wheelchair Transfer Assist: Minimal assist

Bed/Chair/WC Transfer Device: Walker - two wheel

Technique: Stand to sit, Sit to stand, Walking

Comments: Cuing for safety and sequencing as well as fall prevention

Ambulation

Distance (ft): 15

Assistance: Contact guard

Device: Walker - front wheel

Gait Comments: Performed 2x to/from restroom with 1 seated break on toilet. Cuing for safety and sequencing

IP PT Additional Assessments:

PT Functional Outcomes

Row Name	12/17/22 1544
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PT Outcome/Test Tool Selection

Balance / Gait / Mobility (A-L)	6 Clicks - 5 Items
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AM-PAC 6-Clicks Basic Mobility (5 ITEMS) IP Short Form v2

Moving from lying on back to sitting on side of the bed?	3
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Turning over in bed (including adjusting bedclothes, sheets & blankets)?	3
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Sitting down on and standing up from a chair with arms (eg. wheelchair, bedside, commode)?	3
Moving to and from a bed to a chair (including a wheelchair)?	3
To walk in hospital room?	3
AM-PAC Raw Score (5-Items)	15
AM-PAC Standardized T Score (5-Items)	38.88

IP PT Therapeutic Interventions Minutes
Therapeutic Activity Minutes: 16

Assessment

Pt appeared to be self limiting in the treatment this afternoon secondary to reports of feeling dizzy and drowsy due to pain medication. Pt would benefit from continued skilled PT to increase her interdependence and reduce her risk for falls.

Impairments Noted (PT): Balance, Endurance, Mobility, Range of motion, Safety awareness, Pain, Strength
Evaluation/Treatment Tolerance: Limited by fatigue/endurance
Motivation/Participation Level: Engaged
Barriers to Discharge: needs to be Mod Ind with AD for transfers and household distances

PT Goals:

PT Target Goal Date: 12/21/22

Overall Progress Toward PT Goals: Goals reviewed-remain appropriate

Bed Mobility Goal 1 (PT)

Activity/Assistive Device (Bed Mobility Goal 1, PT): bed mobility activities, all

Independence Level/Cues Needed (Bed Mobility Goal 1, PT): independent

Transfer Goal 1 (PT)

Activity/Assistive Device (Transfer Goal 1, PT): transfers, all

Independence Level/Cues Needed (Transfer Goal 1, PT): modified independence

Gait Training Goal 1 (PT)

Activity/Assistive Device (Gait Training Goal 1, PT): gait (walking locomotion), assistive device use

Independence Level (Gait Training Goal 1, PT): modified independence

Distance (Gait Training Goal 1, PT): 300
 ROM Goal 1 (PT)
 ROM Goal 1 (PT): L knee 0-90 degrees
 Strength Goal 1 (PT)
 Strength Goal 1 (PT): pt to complete TKA phase I medbridge Ind
 Safety Awareness Goal 1 (PT)
 Activity (Safety Awareness Goal 1, PT): awareness of need for assistance, insight into deficits/self-awareness, safe use of assistive device/equipment, follow through of safety precautions
 Independence/Cues/Accuracy (Safety Awareness Goal 1, PT): independent

Problem Specific Goal 1 (PT)
 Problem Specific Goal 1 (PT): TKA HEP
 Strategies/Barriers (Problem Specific Goal 1, PT): indep via medbridge

Plan

PT Plan: Continue skilled PT
 PT Treatment Activities/Interventions: Adaptive/assistive equipment, Bed mobility, Gait, Range of motion, Therapeutic activities, Therapeutic exercises, Transfers
 PT Treatment Focus: Body mechanics, Education-patient, Energy conservation, Home exercise program, Posture, Safety
 PT Frequency: 14x/wk

Discharge Dispo/Follow-up Recommendations: Skilled nursing facility
 Post D/C Follow-up Therapy Service Recommendations: Physical Therapy
 D/C Home Planning: pt needs to be Mod Ind with AD for transfers and household distances
 Functional Mobility Equipment Recommended for D/C: None/has all needed equipment
 Adaptive Equipment Recommended for D/C: None/has all needed equipment

Physical Therapy Visit

Electronically signed by Adelaida I Ortiz, PT on 12/17/2022 5:44 PM

Progress Notes by Adelaida I Ortiz, PT at 12/17/2022 9:25 AM

Author: Adelaida I Ortiz, PT	Service: —	Author Type: Physical Therapist
Filed: 12/17/2022 11:53 AM	Date of Service: 12/17/2022 9:25 AM	Status: Signed
Editor: Adelaida I Ortiz, PT (Physical Therapist)		

Inpatient Physical Therapy Treatment Note

Name: Edna Maldonado
MRN/CSN: 13740025 / 5010088545281
DOB / AGE: 8/12/1960 / 62 y.o.

Documentation Type: Treatment

Subjective

Patient Complaints: Pain

Patient Subjective Comments: ok f rtherapy, "I need to use the bathroom first"

Pain Adequately Managed for Therapy Participation: Pain management adequate throughout therapy session

Active Problems:

There are no active Hospital Problems.

Significant Hospital Course Details: 62 y/o female pt admitted to AHC 12/14/22 for elective L TKA by Dr.Homan.(PT 12/14/22)

Precautions

Rehab Precautions/Restrictions: Yes

LE Weight Bearing Status: WBAT

Medical Precautions: Fall Risk

Objective

Family/Caregiver Present: No

Nursing Pre-Treatment Communication: RN ok for therapy

Post Treatment Communication: pt supine on bed, deferred sitting up on the recliner

Activity Tolerance

Endurance: Tolerates 30 min exercise with multiple rests

Early Mobility/Exercise Safety Screen: Proceed with mobilization - No exclusion criteria met

Cognition

Overall Cognitive Status: Within Functional Limits

Arousal / Alertness: Alert, Drowsy

Orientation Level: Oriented X4

Following Commands: Follows all commands and directions without difficulty

Safety Judgment: Decreased awareness of need for assistance, Decreased awareness of need for safety

Awareness of Errors: Assistance required to identify errors made

Deficit Awareness: Fully aware of deficits

Attention Span: Appears intact

PT Exercises

Exercise Activity 1: AP, HS, SAQ, QS

Activity Performance 1: long sitting on bed X10

Therapeutic Activity

Therapeutic Activity 1: HEP education

Therapeutic Activity 2: bed mobility

Therapeutic Activity 3: transfers

Therapeutic Activity 4: fall prevention/home safety

Left Knee Active Range of Motion

L Knee Flexion 0-140: 62

L Knee Extension (0): -9

Static Sitting Balance

Static Sitting-Balance Support: No upper extremity supported

Static Sitting Balance Grade: Good: Able to maintain balance against moderate resistance

Dynamic Sitting Balance

Dynamic Sitting-Balance Support: No upper extremity supported

Dynamic Sitting Balance Grade: Good: Able to sit unsupported & weight shift across midline moderately

Static Standing Balance

Static Standing-Balance Support: Bilateral upper extremity supported

Static Standing Balance Grade: Fair: Able to stand w/o UE support & w/o loss of balance for 1-2 minutes

Static Standing-Comment/Number of Minutes: FWW

Dynamic Standing Balance

Dynamic Standing-Balance Support: Bilateral upper extremity supported

Dynamic Standing Balance Grade: Fair: Able to stand unsupported, weight shift & reach to same side, difficulty crossing midline

Dynamic Standing-Comments: FWW

Postural Control

Postural Control: Within Functional Limits

Functional Mobility:

Modified Rankin Score: Moderately severe disability. Unable to attend to own bodily needs without assistance or unable to walk unassisted.

Bed Mobility

Bed Mobility From: Supine

Bed Mobility Type: To and from

Bed Mobility To: Sit edge of bed

Level of Assistance: Contact guard

Bed Mobility Comments: Min A to ge LLE back on the leg after walking

Transfers

Sit to Stand Assist: Contact guard

Sit to Stand Transfer Device: Walker - front wheel

Comments: vc for hand placement and safety w AD

Ambulation

Distance (ft): 125

Assistance: Contact guard

Device: Walker - front wheel

Other Apparatus: Wheelchair follow

Gait Comments: vc for heel to toe transition and reciprocal gait, required a sitting break 1/2 way 2* fatigue

IP PT Additional Assessments:

PT Functional Outcomes

Row Name 12/17/22 0925

PT Outcome/Test Tool Selection

Balance / Gait / Mobility 6 Clicks - 5 Items
(A-L)

AM-PAC 6-Clicks Basic Mobility (5 ITEMS) IP Short Form v2

Moving from lying on back 3
to sitting on side of the bed?

Turning over in bed 3
(including adjusting
bedclothes, sheets &
blankets)?

Sitting down on and 3
standing up from a chair with
arms (eg. wheelchair,
bedside, commode)?

Moving to and from a bed to 3
a chair (including a
wheelchair)?

To walk in hospital room? 3

AM-PAC Raw Score (5-Items) 15

AM-PAC Standardized T 38.88
Score (5-Items)

IP PT Therapeutic Interventions Minutes

Gait Training Minutes: 30

Therapeutic Activity Minutes: 15

Therapeutic Exercise Minutes: 10

Assessment

Impairments Noted (PT): Balance, Endurance, Mobility, Range of motion, Safety awareness, Pain, Strength

Evaluation/Treatment Tolerance: Limited by fatigue/endurance

Motivation/Participation Level: Engaged

PT Goals:

PT Target Goal Date: 12/21/22

Overall Progress Toward PT Goals: Goals reviewed-remain appropriate

Bed Mobility Goal 1 (PT)

Activity/Assistive Device (Bed Mobility Goal 1, PT): bed mobility activities, all

Independence Level/Cues Needed (Bed Mobility Goal 1, PT): independent

Transfer Goal 1 (PT)

Activity/Assistive Device (Transfer Goal 1, PT): transfers, all

Independence Level/Cues Needed (Transfer Goal 1, PT): modified independence

Gait Training Goal 1 (PT)

Activity/Assistive Device (Gait Training Goal 1, PT): gait (walking locomotion), assistive device use

Independence Level (Gait Training Goal 1, PT): modified independence

Distance (Gait Training Goal 1, PT): 300

ROM Goal 1 (PT)

ROM Goal 1 (PT): L knee 0-90 degrees

Strength Goal 1 (PT)

Strength Goal 1 (PT): pt to complete TKA phase I medbridge Ind

Safety Awareness Goal 1 (PT)

Activity (Safety Awareness Goal 1, PT): awareness of need for assistance, insight into deficits/self-awareness, safe use of assistive device/equipment, follow through of safety precautions

Independence/Cues/Accuracy (Safety Awareness Goal 1, PT): independent

Problem Specific Goal 1 (PT)

Problem Specific Goal 1 (PT): TKA HEP

Strategies/Barriers (Problem Specific Goal 1, PT): indep via medbridge

Plan

PT Plan: Continue skilled PT

PT Treatment Activities/Interventions: Adaptive/assistive equipment, Bed mobility, Gait, Range of motion, Therapeutic activities, Therapeutic exercises, Transfers
 PT Treatment Focus: Body mechanics, Education-patient, Energy conservation, Home exercise program, Posture, Safety
 PT Frequency: 14x/wk

Discharge Dispo/Follow-up Recommendations: Skilled nursing facility
 Post D/C Follow-up Therapy Service Recommendations: Physical Therapy
 D/C Home Planning: pt needs to be Mod Ind with AD for transfers and household distances
 Functional Mobility Equipment Recommended for D/C: None/has all needed equipment
 Adaptive Equipment Recommended for D/C: None/has all needed equipment

Physical Therapy Visit

Electronically signed by Adelaida I Ortiz, PT on 12/17/2022 11:53 AM

Discharge Medications

Unreviewed Medications

	Sig	Disp	Refill	Start	End
ALPRAZolam 0.5 MG tablet Commonly known as: Xanax	TAKE 1 TABLET BY MOUTH THREE TIMES A DAY FOR 3 DAYS AS NEEDED FOR ANXIETY				
aspirin-acetaminophen-caffeine 250-250-65 MG tablet Commonly known as: Excedrin Migraine	1 tablet, Oral, Every 6 hours PRN				
gabapentin 800 MG tablet Commonly known as: Neurontin	TAKE 1 TABLET THREE TIMES DAILY MORNING NOON AND EVENING.				
lactulose 20 gram/30 mL oral solution	20 g, Oral, Daily PRN				
methocarbamol 750 MG tablet Commonly known as: Robaxin	750 mg, Oral, 3 times daily				
pantoprazole 40 MG EC tablet Commonly known as: Protonix Ask about: Should I take this medication?	40 mg, Oral, Daily, Do not crush, chew, or split.				
traMADol (Ultram) 5 mg/mL oral suspension	100 mg, PO, BID, PRN as needed for pain, 0 Refill(s)				

Discharge Summary Notes

No notes of this type exist for this encounter.

No orders found for this encounter